

CLINICAL MULTIPLE CHOICE EVALUATION

LECTURE 9 OF 20: LEG ULCERS, SKIN TEARS & LYMPHOEDEMA



Candidate Instructions

- This exam paper contains **25 strict Multiple Choice Questions** derived from the 2025 vascular curriculum.
- All questions are grounded in your lecture materials covering venous/arterial differentiation, pathophysiology, and assessment criteria.
- Select the single best answer choice (**A, B, C, or D**) for each clinical item.
- The full verification matrix key is isolated at the absolute end of this booklet.

QUESTION 1

How are leg ulcers explicitly defined according to the course presentation?

- A) Deep dermal tear injuries specifically caused by blunt force trauma impacts
- B) Open sores or wounds that occur on the lower extremities, often due to poor circulation
- C) Localized damage to the stratum corneum due to un-blanchable moisture maceration
- D) Profound necrosis occurring after the complete collapse of arterial oxygen perfusion

QUESTION 2

Which of the following descriptions accurately matches the clinical presentation of a 'Venous Ulcer'?

- A) Usually found on the outer side of the ankle; deep, punched-out, and very painful
- B) Usually found below the knee and on the inner ankle; often painless unless infected
- C) Found primarily on the toe tips, displaying a cold, hairless, and pale configuration
- D) Found in deep gluteal folds following prolonged sedentary pressure cycles

QUESTION 3

Arterial ulcers result from inadequate blood supply. What specific visual and symptomatic traits are they associated with?

- A) "Punched out" appearance, painful, deep wounds, cool affected area, pale when elevated
- B) Brown/black stained skin, intense itching, and weeping exudate
- C) Superficial abrasion tracking following mild friction against bedding
- D) Indistinct borders with thickened hyperpigmented skin zones

QUESTION 4

What is the fundamental pathophysiology behind the development of venous disease as described in the slides?

- A) Fatty deposit plaque build-up narrowing the arterial wall pathways
- B) Valve dysfunction causing blood to pool in the lower extremities and fluid to leak into surrounding tissues
- C) Failure of the epidermis to manufacture vitamin D after UV exposure
- D) Excessive production of viscous protein-heavy sweat triggered by emotional strain

QUESTION 5

Atherosclerosis is the causative mechanism for which specific leg condition?

- A) Venous insufficiency
- B) Peripheral arterial disease
- C) Skin tear avulsion
- D) Chronic pruritus

QUESTION 6

The pathophysiology slide states that prolonged ischaemia (reduced blood flow) eventually leads to what clinical outcome?

- A) Rapid proliferation of collagen type III
- B) Tissue necrosis (tissue death)
- C) Increased capillary permeability and oedema
- D) Enhanced epithelial resurfacing

QUESTION 7

What clinical observation suggests a venous origin in the symptom list on Slide 5?

- A) Deep pain when dangling the leg
- B) Brown or black stained skin
- C) Tight, hairless shiny skin
- D) "Punched-out" wound edges

QUESTION 8

Which of the following is considered a primary symptom of arterial disease?

- A) Itchy, hardened skin surfaces
- B) Leg pain at night
- C) General swelling and aching
- D) Scabbing or flaking of the inner ankle

QUESTION 25

What diagnostic classification describes ulcers that display a combination of venous and arterial factors?

- A) Mixed Ulcers
- B) Complex Tears
- C) Secondary Intent wounds
- D) Lymphoedematous lesions

QUESTION 10

What is the significance of the "Valve Dysfunction" mentioned in the pathophysiology of venous disease?

- A) Valves fail to close properly, causing blood to pool in the lower extremities
- B) Valves over-pressurize, pushing arterial blood directly into the bone marrow
- C) Valves become calcified, preventing any oxygenation of the papillary dermis
- D) Valves prevent the return of interstitial fluid to the lymph system entirely

QUESTION 11

What happens when arterial plaque buildup narrows the arterial walls?

- A) It increases blood flow efficiency
- B) It reduces oxygen and nutrient delivery to tissues, causing ischaemia
- C) It triggers immediate tissue regeneration in the proliferative phase
- D) It causes the immediate transition of collagen from type I to type III

QUESTION 12

Which of the following is defined as the cause of venous insufficiency-related inflammation and oedema?

- A) The pooling of blood increases pressure, leading to fluid leakage into surrounding tissues
- B) The rapid production of high-viscosity protein-lipid sweat
- C) The invasion of fungal digestive enzymes that dissolve structural keratin
- D) The formation of arterial fatty plaques that block capillary inflow

QUESTION 13

What clinical sign on Slide 5 is noted as a result of venous disease?

- A) Redness when dangling the leg
- B) Discharge
- C) Cool/cold affected area
- D) Deep, punched-out wounds

QUESTION 14

What explicit statement is made regarding the treatment of mixed ulcers?

- A) They should only be treated as venous ulcers regardless of arterial history
- B) Treatment varies based on the underlying cause, addressing blood flow, wound care, and risk factors
- C) They require surgical amputation of the extremity within 24 hours of diagnosis
- D) They are resistant to all forms of dressing management and should be left uncovered

QUESTION 15

Which statement accurately describes the blood flow changes in arterial disease?

- A) The leg turns red when elevated and pale when dangling
- B) Redness when dangling the leg, turning pale when elevated
- C) No significant color change is associated with positional elevation
- D) The skin becomes permanently darkened and stained brown across all positions

QUESTION 16

What anatomical location is typically associated with Venous Ulcers?

- A) The outer side of the ankle and feet
- B) Around the ankles
- C) The tips of the toes
- D) The deep gluteal sacral crease

QUESTION 17

Why is proper assessment crucial for mixed ulcers?

- A) Because they only respond to antibiotic irrigation
- B) For accurate diagnosis and treatment
- C) Because mixed ulcers have no identifiable underlying vascular cause
- D) Because they are highly contagious to adjacent staff members

QUESTION 18

What symptom does a venous ulcer patient usually experience regarding skin feeling?

- A) Tight, hairless, and smooth feeling
- B) Itchy, hardened skin
- C) Freezing cold feeling
- D) Deep numbness and complete loss of sensory touch

QUESTION 19

What is the appearance of a venous ulcer described as?

- A) "Punched out" and deep
- B) Surface lesions with indistinct borders and discharge
- C) White, dry necrotic craters
- D) Small, black, dry eschar plugs

QUESTION 20

Which of the following is associated with the arterial ulcer appearance?

- A) Scabbing or flaking
- B) Red, yellow, or black sores
- C) Brown stained skin
- D) Excessive weeping exudate

QUESTION 21

What anatomical location is typically associated with Arterial Ulcers?

- A) Inner ankle
- B) Outer side of the ankle, feet, heels, or toes
- C) Upper calf muscles
- D) Sacral prominence

QUESTION 22

What causes venous ulcers specifically?

- A) Insufficient blood return to the heart due to vein dysfunction
- B) Blocked arteries preventing oxygen reach
- C) Fungal digestive enzyme consumption
- D) Prolonged vertical pressure from wheelchairs

QUESTION 23

What causes arterial ulcers specifically?

- A) Fluid leakage into tissues
- B) Reduced blood flow due to inadequate nutrient-rich blood reaching tissue
- C) Infection via *Staphylococcus aureus*
- D) Chronic inflammation from poor hygiene

QUESTION 24

Which of the following is associated with the symptom of a venous ulcer?

- A) Cool or cold affected area
- B) Aching
- C) Punched out appearance
- D) Redness when dangling the leg

QUESTION 25

Which of the following is an accurate statement about mixed ulcer treatment?

- A) Treatment is always identical to arterial ulcer management
- B) Addressing blood flow issues is essential for healing
- C) Mixed ulcers cannot be healed
- D) They do not require wound care

OFFICIAL EXAM ANSWER KEY MATRIX

Verify student choice selections strictly using this validation matrix block.

Q1	Q2	Q3	Q4	Q5	Q6	Q7	Q8	Q9	Q10
B	B	A	B	B	B	B	B	A	A

Q11	Q12	Q13	Q14	Q15	Q16	Q17	Q18	Q19	Q20
B	A	B	B	B	B	B	B	B	B

Q21	Q22	Q23	Q24	Q25
B	A	B	B	B